

EMS**Littell County Emergency Medical Services**

12 Station Road, Littell, RI 02898 · Tel 555-0110 · Fax 555-0111

PREHOSPITAL CARE REPORT — EMS RUN SHEET RUN #24-08812

UNIT <i>Rescue 4.</i>	DATE <i>09/04/2023</i>	NATURE OF CALL <i>MVC — entrapment?</i>
DISPATCHED <i>18:48</i>	ON SCENE <i>18:54</i>	DEPART / AT HOSP <i>19:12 / 19:24</i>
PATIENT NAME: <i>Dax Vance</i> DOB: <i>04/12/2001</i> GCS: <i>14</i>		VITALS (INITIAL) BP: <i>138/88</i> HR: <i>110</i> RR: <i>20</i> SPO2: <i>97</i> PAIN: <i>10</i>

SAFETY EQUIPMENT
Seat belt: ☐ Yes ☐ No ☐ Unknown Airbag deployed: ☒ Yes ☐ No Helmet: ☐ N/A ☒ —

NARRATIVE
Dispatched to MVA. Arrived to find 2-vehicle T-bone collision with heavy side-impact damage to sedan. Driver side crushed. Patient Dax Vance (22M) was trapped inside, requiring mechanical jaws-of-life extraction. Patient was conscious, highly combative, and in severe distress. Left upper extremity displays obvious gross compound deformity of humerus with active bleeding and bone fragments visible. Lower left arm/wrist also displays spiral deformity with severe swelling. Patient complains of severe neck and lower back pain, head pain and left knee pain. Heavy odor of alcohol detected on Patient's breath. Patient admitted: 'I had 4 beers before driving. I shouldn't have been on the road.' Left arm splinted, wound dressed. Cervical collar applied. Spine boarded. Transported Code 3 to Littell County Emergency Hospital. Left forearm pulses faint.

INTERVENTIONS ☒ C-collar ☒ Splint ☒ O2 ☐ IV ☐ Meds ☒ Assess	CREW / SIGNATURE <i>Paramedic James Vance, NREMT-P</i>
--	--

INPATIENT ADMISSION — FACE SHEET

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
----------------------	-------------------	-------------------	-------------------------------

PATIENT DEMOGRAPHICS		ADMISSION	
Name	Dax Vance	Admit Date / Time	09/04/2023 2023-09-04 19:28
Date of Birth / Sex	04/12/2001 / Male	Admission Type	Emergency
MRN / Account	940-2281-A / ACT-99214-04	Attending	Dr. Myrtle Turner, MD, FACS
Address	482 Poplar Street, Apt 3B, Littel, RI 02898	Admitting	Dr. Elizabeth Foster, MD
Phone	555-0144	Unit / Bed	—
Employer	Littel Lumber Yard (Warehouse Associate)	Mode of Arrival	Ambulance
Primary Care	Dr. Alistair Finch	Disposition	Inpatient Admission
GUARANTOR		DIAGNOSES	
Guarantor	Dax Vance	Admitting DX	Left humerus compound fracture, Left radius spiral fracture, Cervical spine injury, Left knee contusion
Financial Class	Commercial Auto/Health	Principal DX	Closed fracture of shaft of left humerus, open fracture of distal radius, acute neck pain, alcohol intoxication
EMERGENCY CONTACT		INSURANCE	
Name	Sylvia Vance (Mother)	Primary	Sentinel Mutual Auto Insurance Member SM-4482-9921 · Group AUTO-MEDPAY
Phone	555-0199	Secondary	Aetna Care Health Plan Member W882931102 · Group CT-IND-90
		ALERTS	
		Allergies	NKDA
		Code Status	Full code

EMERGENCY DEPARTMENT — TRIAGE ASSESSMENT

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/04/2023

TRIAGE TIME	19:30	ESI LEVEL	2 — Emergent
-------------	-------	-----------	--------------

ARRIVAL MODE	EMS — ncfd
--------------	------------

CHIEF COMPLAINT	MVA, left arm deformity, severe neck and back pain, left knee pain, head pain
-----------------	---

BP	HR	RR	SPO2	TEMP	PAIN (0-10)
142/90	114	22	96	98.4	10

ALLERGIES

NKDA

HOME MEDICATIONS (PER PATIENT)

None

TRIAGE SCREENING

Fall risk (Morse)

Suicide screen (C-SSRS)

Alcohol use screen (CAGE brief)

Domestic safety screen

Last tetanus

High — 65 (recent trauma, analgesia anticipated)

Negative

Patient reports 2 beers at lunch today; odor noted by EMS — serum ETOH ordered

Negative

Unsure — Tdap administered per protocol

Dr. Elizabeth Foster

Dr. Elizabeth Foster, MD — Emergency Medicine

• NPI 1992038171 • Signed 09/04/2023



Littel Inc Emergency Hospital

450 Littel Rest Road, Littel, RI 02898 · Tel 555-0120 · Fax 555-0121

LABORATORY REPORT — FINAL

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
----------------------	-------------------	-------------------	-------------------------------

ACCESSION	LAB-2023-0904-8	SPECIMEN	Blood
COLLECTED	09/04/2023 2023-09-04 19:45	ORDERING PROVIDER	Dr. Elizabeth Foster, MD
RECEIVED	09/04/2023 2023-09-04 19:55	REPORTED	09/04/2023 2023-09-04 20:22

EMERGENCY TOXICOLOGY AND METABOLIC PANEL

TEST	RESULT	UNITS	REFERENCE RANGE
Serum Ethanol	110 (H)	mg/dL	< 10
WBC	14.2 (H)	10 ³ /uL	4.5-11.0
Hemoglobin	12.8 (L)	g/dL	13.8-17.2
Creatinine	1.1	mg/dL	0.6-1.2

H = above reference · L = below reference · A = abnormal — reviewed and released by laboratory staff. Performing lab: Littel Inc Emergency Hospital Clinical Laboratory, CLIA 36D-0000000 (synthetic).

ED PHYSICIAN NOTE - CLINICAL ASSESSMENT

PATIENT	DOB	MRN	DATE OF SERVICE
Dax Vance	04/12/2001	940-2281-A	09/04/2023

Presenting History

Patient is a 22-year-old male driver of a sedan involved in high-speed, multi-vehicle side-impact MVA at 18:45 tonight. Requires extraction due to vehicle frame distortion. Patient is alert but cooperative only intermittently, demonstrating mild combativeness and an obvious odor of alcohol. Confirmed consumption of 4 beers pre-crash. Complaining of 10/10 severe pain in left arm, severe cervical neck pain, lower back discomfort, and a heavy headache.

Physical Examination

- **HEENT:** Active 2cm superficial laceration over left parietal scalp, mild tenderness to palpation, pupils equal and reactive.
- **Neck:** Severe tenderness over C4-C7 vertebrae. Prominent muscle spasm in bilateral trapezius. Pain aggravated by any passive movement. Cervical collar maintained.
- **Cardiovascular / Pulmonary:** Tachycardic but regular. Lungs clear bilaterally.
- **Left Upper Extremity:** Gross deformity of left humerus mid-shaft with an open skin puncture measuring 2.5cm, bone fragment visible. Active bleeding present but partially controlled with sterile pressure dressings. Severe swelling and visible spiral deformity of distal radius. Left radial pulse is 1+ (weak) and capillary refill is delayed at 3.5 seconds. Distal sensation is moderately decreased in the radial nerve distribution.
- **Left Lower Extremity:** Focal bruising and tenderness over anterior left knee, range of motion guarded.
- **Back:** Mild paraspinal lumbar tenderness, no step-offs.

Assessment & Plan

1. Left humerus mid-shaft open/compound fracture.
2. Left distal radius spiral fracture.
3. Acute cervicalgia with high suspicion of ligamentous or disc injury.
4. Acute scalp laceration.
5. Ethanol intoxication. Consult Orthopedic Surgery (Dr. Myrtle Turner) immediately for emergency open reduction and internal fixation (ORIF). Initiate IV antibiotics (Ancef) and aggressive pain management. Obtain emergency X-rays of left arm, C-spine, and lumbar spine.

**DIAGNOSTIC IMAGING REPORT**

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
----------------------	-------------------	-------------------	-------------------------------

EXAMINATION	X-ray Left Upper Extremity (Humerus, Forearm)
ACCESSION	RAD-2023-8812 · Exam date 09/04/2023
INTERPRETING MD	Dr. Elizabeth Foster, MD

TECHNIQUE

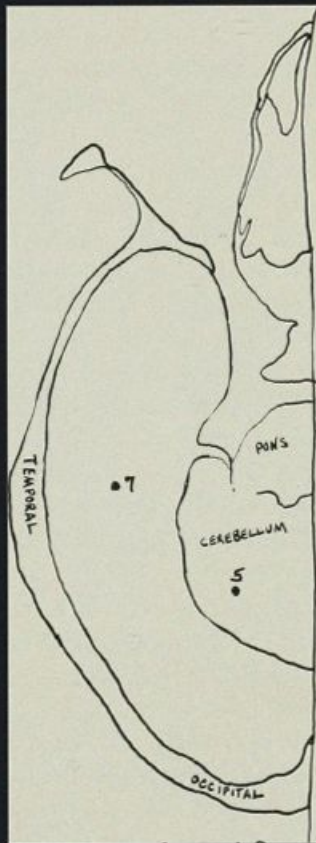
Three-view radiographs of the left humerus and left forearm.

FINDINGS

There is an displaced, mid-shaft fracture of the left humerus with a lateral bone fragment piercing through adjacent subcutaneous tissue (open compound presentation). Substantial overriding of fracture fragments by 2.2 cm. Additionally, there is a complex spiral fracture of the distal one-third of the radius, with intra-articular extension. Mild posterior displacement of the distal radial fragments. Moderate swelling in surrounding soft tissues.

IMPRESSION

1. Highly unstable compound mid-shaft humerus fracture with significant overriding. 2. Displaced spiral fracture of the distal radius with intra-articular extension.



DAX VANCE · 940-2281-A · 09/04/2023 · RAD-2023-8812
Multiple spiral and compound fractures of the left humerus shaft and distal radius

Dr. Elizabeth Foster

Dr. Elizabeth Foster, MD — Emergency Medicine
NPI 1992038171 · Signed 09/04/2023

Electronically signed. Images archived on PACS under accession RAD-2023-8812.

DIAGNOSTIC IMAGING REPORT

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/04/2023
----------------------	-------------------	-------------------	-------------------------------

EXAMINATION	X-ray Cervical Spine
ACCESSION	RAD-2023-8813 · Exam date 09/04/2023
INTERPRETING MD	Dr. Elizabeth Foster, MD

TECHNIQUE

AP, lateral, and odontoid views of the cervical spine.

FINDINGS

There is narrowing of the C5-C6 intervertebral disc space with osteophytic lipping. Mild retrolisthesis of C5 on C6. A prominent 6mm posterior/paracentral disc bulge is visualized at the C5-C6 level, compressing the anterior thecal sac. There is moderate prevertebral soft tissue swelling from C4 through C6. No acute vertebral body fractures identified.

IMPRESSION

1. C5-C6 disc narrowing and significant posterior disc bulge measuring 6mm, producing moderate central canal narrowing. 2. Prevertebral soft tissue swelling, consistent with acute flexion/extension trauma. Clinical correlation with prior records is advised as the onset date of the bulge is inconclusive.



Dr. Elizabeth Foster

Dr. Elizabeth Foster, MD — Emergency Medicine

NPI 1992038171 · Signed 09/04/2023

Electronically signed. Images archived on PACS under accession RAD-2023-8813.